

REVENUE CYCLE
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Claim Appeal Writing Template

A practical, payer-adaptable toolkit for provider claim appeals

Inside this toolkit

Universal appeal letter, denial-specific language guides, evidence checklist, quality review checklist, submission log, and follow-up tracker.

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How to Use This Toolkit

Start with the payer instructions

Before writing, confirm the correct submission route, appeal level, deadline, required form, address or portal, representative authority, and documentation requirements. The payer notice and provider contract control.

1. Validate the adjudication

Review the ERA/EOB, group code, CARC, RARC, line-level details, claim history, payer policy, and contract.

2. Choose the correct route

Use a corrected claim for supported billing-data changes. Use reconsideration or appeal to dispute a processed decision. Follow payer terminology.

3. Build the argument

State the decision, disputed service, factual basis, policy or contractual basis, and requested remedy.

4. Attach evidence

Include only relevant records, authorization, proof of timely filing, remit, claim, policy excerpt, and other required items.

5. Quality review and submit

Check accuracy, deadline, privacy, attachments, signature, and submission confirmation. Set a follow-up date.

Core Writing Principles

- Be specific about the claim, service line, denial reason, and requested outcome.
- Use factual, respectful language. Avoid emotional statements and unsupported conclusions.
- Connect each claim to documentation, policy, authorization, contract terms, or adjudication evidence.
- Do not add, alter, or infer coding, authorization, clinical facts, or documentation that are not supported by the record.
- Protect patient information and use only approved secure submission channels.

Universal Provider Claim Appeal Letter

Template instruction

Replace all bracketed text. Delete optional sections that do not apply. Keep the final letter concise and attach a clearly indexed evidence packet.

Date: [Month Day, Year]

Submission method: [Payer portal / fax / certified mail / other approved route]

Payer appeal department: [Department name]

Payer address or portal reference: [Address / portal confirmation]

Claim and Provider Information

Patient name	[Name]	Member ID	[ID]
Claim number / ICN	[Number]	Date(s) of service	[Date(s)]
Provider name	[Name]	Billing NPI / TIN	[NPI / TIN]
Service(s)	[Description / code where permitted]	Billed / disputed amount	[\$]
Denial or adjustment	[Group code, CARC, RARC, payer message]	Appeal level	[Initial / reconsideration / level]

Subject

Request for [reconsideration / appeal / payment review] of claim [claim number] for date(s) of service [date(s)].

Dear Claims Review Team,

We respectfully request review of the above-referenced claim, which was [denied / reduced / underpaid] for “[insert payer reason].” After reviewing the remittance, claim history, supporting documentation, and applicable [payer policy / authorization / contract provision], we believe the claim should be [reprocessed / paid / adjusted] for the reasons summarized below.

Basis for Appeal

[State the relevant facts in chronological order. Explain exactly why the payer decision should be changed. Identify the supporting evidence by attachment number. Example: “Authorization [number] was valid for the billed service, provider, units, and date of service. See Attachments 3 and 4.”]

Requested Resolution

Please [reprocess the claim / overturn the denial / apply the contracted rate / issue the additional payment] and provide an updated remittance reflecting the determination. If additional information is required, please contact [name / department] at [secure contact information].

Sincerely,

Authorized representative: [Name and title]

Provider / organization: [Name]

Phone / secure email: [Contact]

Appeal Argument Builder

Use this page before drafting

A strong appeal answers five questions: What happened? Why is the decision incorrect? What evidence proves it? What rule supports the request? What exact remedy is requested?

1. Decision being disputed

The claim was [denied / reduced / underpaid] for [exact payer reason, code, and remark].

Draft notes: _____

2. Correct factual record

The record shows [authorization, eligibility, timely submission, coding support, documentation, contract term, or payer history].

Draft notes: _____

3. Evidence

Attachment [#] demonstrates [specific fact]. Attachment [#] supports [specific service or requirement].

Draft notes: _____

4. Policy or contractual basis

Under [payer policy title/version, authorization, provider contract section, or applicable official guidance], [state the relevant requirement in your own words].

Draft notes: _____

5. Requested remedy

We request [reprocessing, denial reversal, payment at the contracted rate, corrected patient liability, or written rationale].

Draft notes: _____

Tone Check

- ☐ The letter is factual and professional.
- ☐ Each assertion is supported by a cited attachment.
- ☐ The request is specific and operationally clear.
- ☐ No unsupported clinical, coding, legal, or contractual conclusion is included.

Denial-Specific Language Guides

Important

These are adaptable writing prompts, not payer-specific appeal language. Use only facts supported by the claim record and current payer requirements.

Medical Necessity

The service was denied for lack of medical necessity. The clinical record documents [symptoms, diagnosis, prior treatment, failed conservative measures, risk, and clinical rationale]. The documentation addresses the relevant criteria in [policy title/version]. See Attachments [#]. We request clinical reconsideration and reprocessing.

Suggested evidence: Clinical note, order, diagnostic results, therapy history, payer medical policy, authorization, peer review or physician rationale where permitted.

Authorization or Notification

The payer indicated that authorization or notification requirements were not met. Authorization [number] was issued for [service], [units], [provider/site], and [date range], and it applies to the billed service. Alternatively, [documented emergency / payer system issue / approved retro-review basis] supports an exception. See Attachments [#].

Suggested evidence: Authorization record, portal confirmation, call reference, order, schedule, claim, emergency documentation, payer requirement.

Timely Filing

The claim was first transmitted on [date], within the applicable filing period. The attached [clearinghouse acceptance / payer acknowledgment / certified mail / portal receipt] confirms receipt or acceptance. Subsequent activity occurred on [dates]. We request reversal of the timely-filing denial and reprocessing.

Suggested evidence: Filing-limit source, 999/277CA or clearinghouse report, payer acknowledgment, claim history, corrected-claim history, correspondence.

Eligibility or COB

Coverage was active for the date of service, and the payer order was [primary / secondary] based on [eligibility response, COB update, primary EOB, or other evidence]. The attached documents support the correct coverage and sequence. We request that COB be updated and the claim reprocessed.

Suggested evidence: Eligibility response, insurance card, COB questionnaire, primary EOB/ERA, accident/employment details where applicable.

Coding, Modifier, or Bundling

The billed service and modifier are supported by the documentation and were reported according to the applicable coding and payer rules for the date of service. The services were [distinct / separately reportable / outside the global service] because [documented factual basis]. See Attachments [#].

Suggested evidence: Claim detail, operative or visit note, coding review, payer edit policy, relevant code-set guidance held under proper license.

Underpayment or Contractual Variance

The allowed amount or payment does not match the applicable contracted methodology for [service, modifier, place of service, or fee schedule period]. Based on [contract section / fee schedule / payment policy], expected reimbursement is [\$], compared with actual reimbursement of [\$], resulting in a variance of [\$].

Suggested evidence: Remit, contract excerpt, fee schedule, expected-payment calculation, claim, prior payer correspondence.

Duplicate Claim

The claim is not a duplicate because [different service, separate encounter, repeat service, corrected-claim indicator, distinct units, or prior claim reversal]. The attached claim history and documentation distinguish the billed service from the previously processed claim.

Suggested evidence: Both claim records, ICN/DCN, remit, corrected-claim indicator, documentation, modifiers if supported.

Noncovered Service or Benefit Exclusion

The denial states that the service is not covered; however, [benefit verification, plan material, authorization, payer correspondence, or policy] indicates coverage for the service under the applicable conditions. We request benefit review and reprocessing.

Suggested evidence: Benefit verification, plan document, authorization, payer correspondence, required patient notice, clinical documentation.

Appeal Evidence Packet Checklist

Required item	Included?
Payer-required appeal, reconsideration, dispute, or cover form	☐ Yes ☐ N/A
Appeal letter signed by an authorized representative	☐ Yes ☐ N/A
Copy of the claim and relevant claim lines	☐ Yes ☐ N/A
ERA/EOB/remittance showing the decision, group code, CARC, and RARC	☐ Yes ☐ N/A
Payer denial or determination letter	☐ Yes ☐ N/A
Relevant clinical records, order, report, and supporting documentation	☐ Yes ☐ N/A
Authorization, referral, notification, or portal confirmation	☐ Yes ☐ N/A
Eligibility, benefit, or COB evidence	☐ Yes ☐ N/A
Proof of timely filing or claim acceptance	☐ Yes ☐ N/A
Corrected-claim history and prior payer correspondence	☐ Yes ☐ N/A
Applicable payer policy title/version or contract excerpt	☐ Yes ☐ N/A
Expected reimbursement calculation for underpayment disputes	☐ Yes ☐ N/A
Representative authorization or waiver form when required	☐ Yes ☐ N/A
Attachment index with page count	☐ Yes ☐ N/A
Secure submission confirmation, fax report, portal receipt, or tracking number	☐ Yes ☐ N/A

Attachment Index

Attachment	Document	Purpose	Pages
1	[Document name]	[Fact supported]	[Page range]
2	[Document name]	[Fact supported]	[Page range]
3	[Document name]	[Fact supported]	[Page range]
4	[Document name]	[Fact supported]	[Page range]
5	[Document name]	[Fact supported]	[Page range]
6	[Document name]	[Fact supported]	[Page range]

7	[Document name]	[Fact supported]	[Page range]
8	[Document name]	[Fact supported]	[Page range]

Pre-Submission Quality Review

Required item	Included?
Correct payer, plan, appeal department, and submission route confirmed	☐ Yes ☐ N/A
Correct appeal level and deadline confirmed from current payer instructions	☐ Yes ☐ N/A
Patient, member, claim, provider, and date-of-service information matches the record	☐ Yes ☐ N/A
Exact denial or payment issue stated accurately	☐ Yes ☐ N/A
Corrected claim versus appeal route selected appropriately	☐ Yes ☐ N/A
Argument is concise, factual, and supported by evidence	☐ Yes ☐ N/A
Every attachment referenced in the letter is included and readable	☐ Yes ☐ N/A
No unnecessary sensitive information is included	☐ Yes ☐ N/A
Coding, modifier, diagnosis, authorization, and clinical statements are supported	☐ Yes ☐ N/A
Contract or policy version applies to the date of service	☐ Yes ☐ N/A
Requested outcome is specific	☐ Yes ☐ N/A
Authorized signature or representative documentation is included	☐ Yes ☐ N/A
Submission confirmation will be retained	☐ Yes ☐ N/A
Next follow-up date and owner are assigned	☐ Yes ☐ N/A

Appeal Submission and Follow-Up Log

Submitted	Method / confirmation	Appeal level	Amount	Due / follow-up	Outcome
[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]
[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]
[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]
[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]
[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]

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[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]
[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]
[Date]	[Portal / fax / mail / ref]	[Level]	[\$]	[Date]	[Pending / outcome]

Reference Notes and Educational Disclaimer

Practical distinctions

A corrected claim changes supported billing information on the original claim. An appeal or reconsideration disputes a processed coverage or payment decision. Payers may use different terminology and workflows, so always follow the current payer instructions and provider contract.

For remittance review, consider the claim adjustment group code, Claim Adjustment Reason Code (CARC), Remittance Advice Remark Code (RARC), claim-line data, adjustment amount, and associated payer message together.

Official references

- **CMS Medicare Fee-for-Service: Second Level Appeal - Reconsideration:**
<https://www.cms.gov/medicare/appeals-grievances/fee-for-service/second-level-appeal>
- **CMS Medicare Claims Processing Manual, Chapter 29 - Appeals of Claims Decisions:**
<https://www.cms.gov/regulations-and-guidance/guidance/manuals/downloads/clm104c29.pdf>
- **CMS Health Care Payment and Remittance Advice:** <https://www.cms.gov/medicare/coding-billing/electronic-billing/health-care-payment-remittance-advice>
- **X12 Remittance Advice Remark Codes:** <https://x12.org/codes/remittance-advice-remark-codes>

Educational disclaimer

This toolkit is for education and workflow support only. It is not legal, clinical, coding, accounting, contractual, compliance, or payer-specific advice. Appeal rights, deadlines, forms, evidence requirements, authorization rules, and submission channels vary by payer, plan, jurisdiction, provider contract, and appeal level. Verify current requirements before submission.

Final writing reminder

The strongest appeal is not the longest. It clearly identifies the decision, proves the relevant facts, connects the facts to the applicable requirement, includes the necessary evidence, and asks for a specific resolution.